Leading a session: Grief in the community



For people trained in psychological first aid, with a supervisor and known referral routes.

Before the talk, read the facilitator pages from page 3: the words, your limits and

Immediate danger? Pause the session, stay with the person and get urgent help now. Do not leave them alone. Stay until a health worker or responder takes over; feeling calmer is not the end point. Full procedure and contacts: page 2.



Poster A3



Leaflet, tri-fold

Your materials

Poster · a shared focus
For a group: open the discussion, invite observations and return to the main message. Make sure everyone can see; do not read every line aloud.

Leaflet · detail to take home
For a question or a closer look, and to keep or share. Open only the relevant section and offer to read it together.

Use one or both, in any order. Focus on what people need, not on finishing both materials.

Before people arrive

- ☐ Talk with community leaders first about whether, when and where a group talk on grief is welcome. Hang the poster where all can see it; bring enough leaflets.
- ☐ Fill in and check the help contacts on page 2, including where verified tracing information for missing relatives is available. Plan time after the session for private talks.
- ☐ Plan language and access: simple words, large print, more time, or a sign language interpreter if needed and available. Offer alternatives if the picture or text is not accessible.

The session

about 15 minutes · do not rush someone or delay urgent help to finish the plan

Aim: people know grief is a normal response, how to be there for someone who is grieving, and when and where to get more help.

1 Welcome and explain the purpose

2 min · either material

DO Introduce yourself and the topic. Joining is voluntary. Say clearly that nobody will be asked about their own loss in the group.

SAY *"Many families here have lost someone. Today we talk about grief in general, and how we can be there for each other."*

2 Look at the picture together

3 min · poster if useful

DO Point to Asha and Satya, then to the neighbours around them. Welcome different answers without judging or debating beliefs. With the leaflet only, use its cover.

ASK *"What do you see the neighbours doing?"*

SAY *"People grieve in different ways. These feelings can be part of grief, and you can ask for support at any time."*

3 Explore ways of being there

4 min · move between materials as needed

DO Go through the ways of being there shown in the picture: stay close; listen, or sit quietly; help with daily tasks; let them grieve in their own way.

SAY *"You do not need to give names or share anyone's personal experience."*

ASK *"What are some ways people here can offer company or practical help to a grieving family?"*

4 Missing relatives and the route to help

3 min · relevant help section

DO Mention gently that hope and grief can come together when someone is missing. Say where verified tracing information is. Explain the locally checked help contact; use page 2 if concern arises.

ASK *"If someone needs help today, where could they go?"*

5 Check understanding and close

3 min · offer the leaflet to keep

DO Offer the leaflet and show "Children grieve too" and "When to ask for help". Say you are available to talk privately after the session.

ASK *"In your own words, what is one thing you would share with someone at home?"*

Safety, referral and worker boundaries

Get urgent help now if someone:

SIGNS

- Talks about suicide, or about harming themselves or someone else
- Cannot keep themselves safe, or cannot care for a child in their care
- Seems very confused, or not aware of where they are

WHAT TO DO

1. Stay with the person. Do not leave them alone.
2. Get urgent help now: a health worker, responder or the emergency service in this area.
3. It is okay to ask directly about suicide. Asking does not put the idea in their mind.
4. Stay until a health worker or responder takes over. Feeling calmer is not the end point.

If someone becomes upset

- Stay calm and stay close.
- Offer a quieter place away from the group, if they wish.
- Do not force them to talk. Ask what they need now.
- Stay as long as they want, or until someone they trust is with them.

PFA: Look, Listen and Link

Look: safety, urgent needs, people who are very distressed
Listen: ask about needs and concerns; help them feel calm
Link: basic needs, services, information, loved ones

Talk privately and link to further help if

- Feelings get worse, do not improve, or make daily life very hard
- They are not able to look after themselves or their children
- A body problem worries them: pain, fever, injury, or not eating or sleeping for days

With the person's agreement: explain the options, go with them if you can, and share only what is needed.

Do

- Use simple words and respect silence
- Welcome different ways of grieving without judging or debating beliefs
- Ask what the family chooses
- Offer a private conversation after the session

Do not

- Ask anyone about their loss in front of the group
- Say “be strong”, compare losses, or set a time for grief to end
- Assume a missing person has died, or push for “closure”
- Prescribe prayer, rites, touch or talking
- Talk about grief as a set of stages

Do not endorse practices that cause harm or are forced on someone. If there is a safety or protection concern, follow your safeguarding procedure and seek supervision.

Help contacts for this area

fill in and check before the talk

Health worker or facility _____

Urgent help _____

Tracing information for missing relatives _____

Other support _____



Sources: WHO, Psychological First Aid: Guide for Field Workers (2011); WHO, Doing What Matters in Times of Stress (2020); IASC, Guidelines on MHPSS in Emergency Settings (2007); WHO/UNHCR, mhGAP Humanitarian Intervention Guide (2015); IFRC Psychosocial Centre, Supporting grieving family members and friends; IFRC Psychosocial Centre, Working with people experiencing ambiguous loss. This material draws on these publications; it was not created by WHO, IASC or IFRC, which are not responsible for its content.

What this material is about

The poster and leaflet use everyday words, so that people can talk about grief and ask for support without clinical labels. This page gives the meaning behind those words and the limits of your role; page 4 holds the clinical terms, for reference only.

Word key

ON THE POSTER OR LEAFLET	WHAT WE MEAN	HOW TO HANDLE IT IN THE TALK
"There is no right way to grieve"	Grief is a normal response to loss; it has no fixed stages or timetable	Do not present grief as stages; do not say when it should end
"When a loved one is missing, hope and grief can be felt at the same time"	Ambiguous loss: loss without confirmation of death	Never presume death; do not use "closure"; link to verified tracing information
"Grief is making daily life hard" (help message)	Grief that stays very intense and impairs functioning may need assessment; this is decided by a health worker, not in the group	Do not label anyone's grief as abnormal or complicated; offer early help without waiting

Grief, and when to look further

Grief

A normal response to loss, with no fixed stages or timetable. Mourning follows each family's culture and beliefs. Crying, missing the person, anger, guilt, tiredness and poor sleep can all be part of it.

Early help, at any time

If grief makes eating, sleeping or caring for oneself or others very hard, or the person asks for support, link them to a health worker. Do not wait for six months.

Get help now

If there is danger, follow page 2 at once: stay with the person and get urgent help.

Keep these apart

- **Grief** is a normal response to loss, not an illness.
- **Ambiguous loss** describes loss without certainty, such as a missing relative. It is a description, not a diagnosis.
- **A diagnosis** describes a lasting pattern that impairs daily life, assessed by a trained health worker.

Your limits

- You do not judge whether someone's grief is "too much" or "too long".
- You do not speak for a religion or tradition; the family chooses how to mourn.
- You do not give news about missing people; you link to verified tracing information.

Your role

This guide supports a session; it does not replace psychological first aid training. Use your training, supervisor and known referral routes.

Look after yourself too: take breaks, talk with your supervisor or a colleague, and ask for support when you need it.



Talking privately after a loss

- Introduce yourself and ask permission. Explain what you can and cannot offer.
- Help with one need the person chooses.
- Agree any follow-up only if it is possible and wanted; do not promise a visit or call you cannot make.
- Listen, or accept silence. Acknowledge the loss.
- If a relative is missing, link to verified tracing information. Never promise an answer or a date.

Sources: WHO, Psychological First Aid: Guide for Field Workers (2011); WHO/UNHCR, mhGAP Humanitarian Intervention Guide (2015); WHO, mhGAP module: Assessment and management of conditions specifically related to stress (2013), bereavement; IFRC Psychosocial Centre, Supporting grieving family members and friends; IFRC Psychosocial Centre, Working with people experiencing ambiguous loss.

Clinical terms you may hear

For reference only. Do not use these terms with participants, and do not diagnose. They help you understand what a health worker may say, and why early help never waits for a time limit.

TERM OR SITUATION	WHAT THE FACILITATOR NEEDS TO KNOW	SOURCE
Grief	A normal response to loss, with no fixed stages or timetable; mourning follows each family's culture and beliefs.	WHO mhGAP stress module (2013), bereavement; IFRC Psychosocial Centre, Supporting grieving family members and friends
Ambiguous loss	Loss without certainty, for example a missing relative. Hope and grief can exist together. It is a description, not a diagnosis; never presume death and do not push for "closure".	IFRC Psychosocial Centre, Working with people experiencing ambiguous loss
Prolonged grief disorder	After the death of a partner, parent, child or other person close to the bereaved: a persistent and pervasive grief response, with longing for or preoccupation with the person who died, that lasts atypically long (more than six months at a minimum in ICD-11; at least twelve months for adults in DSM-5-TR), clearly exceeds the person's social, cultural or religious norms, and causes significant impairment. Assessed only by a trained health worker.	WHO, ICD-11, 6B42; American Psychiatric Association, DSM-5-TR (2022)
When to look further	Grief that makes eating, sleeping or caring for oneself or others very hard, or a person asking for support: early help at any time. Do not wait for six months.	WHO mhGAP stress module (2013)
Medicines	WHO advises that benzodiazepines should not be offered to bereaved children, adolescents or adults who do not meet the criteria for a mental disorder. Facilitators never suggest medicines; they link the person to a health worker.	WHO, Guidelines for the management of conditions specifically related to stress (2013)
"Trauma", when a participant uses the word	Do not correct the word and do not start a discussion about trauma. Ask what the person means: "When you say trauma, what do you mean? What do you notice in yourself?" People often use the word for a specific fear or change, for example "I am afraid to go back home because the flood might come again" or "because an earthquake might come". Such fear is a common reaction after a frightening or potentially traumatic experience. Respond to what the person describes, and use the early-help route if it continues, gets worse or makes daily life hard.	Facilitation practice for this series; listening as in WHO, Psychological First Aid (2011)

Practice cards

Examples of what people may say and what you can do, step by step. They take place in a private conversation, not in front of the group. The words are an illustration: use your own, follow the person's lead, and use page 2 at once if there is any urgent sign.

WHEN A RELATIVE IS MISSING

"My son is still missing. People say he is gone, but I know he will come back."

Poster and leaflet: when a loved one is missing, hope and grief can be felt at the same time.

1 Listen without taking a side

DO Do not agree or disagree about whether he has died. Never presume death, and do not talk about "closure".

2 Name what they may feel

SAY *"When someone is missing, hope and grief can be felt at the same time. Both are natural."*

3 Link to verified information

DO Give the verified tracing contact. Never promise an answer or a date.

4 Ask about today

ASK *"What would help you today while you wait?"*

DO Help with one need the person chooses.

WHEN SOMEONE WORRIES ABOUT HOW THEY GRIEVE

"I have not cried once. Maybe something is wrong with me."

Poster: there is no right way to grieve.

1 Name the variety of grief

SAY *"There is no right way to grieve. People grieve in different ways, and there is no set time."*

2 Invite, do not push

ASK *"Would you like to tell me a little about how you have been remembering her, in your own way?"*

DO Accept a "no". Do not prescribe crying, rites, prayer or talking.

3 Keep the route to help open

DO Offer a health worker if grief makes it very hard to eat, sleep or care for themselves or others, or if they want support at any time.

WHEN A NEIGHBOUR ASKS HOW TO HELP

"I do not know what to say to her. I am afraid of saying the wrong thing."

Poster: being there for someone who is grieving.

1 Reassure the helper

SAY *"You do not need special words. You can stay close, listen, or simply sit quietly with her."*

2 Suggest practical help

DO Ask her what would help today, for example bringing food, collecting water or helping with a daily task, if she wants it.

3 Name what to avoid

DO Avoid saying "be strong", comparing losses, or telling her when she should feel better.

4 Know where help is

DO Tell the neighbour where help is. If they are worried about her safety, use page 2.